

Discharge against medical advice

Procedure

TARGET AUDIENCE and SETTING

This procedure applies to all medical employees.

PURPOSE

This procedure applies in the following setting:

This procedure is applicable in circumstances where a patient and/or their family members expresses an intent or desire to discharge against medical advice, and the clinician considers this to pose a risk to the patient's safety.

PRECAUTIONS / CONTRAINDICATIONS

1. Under the Medical Treatment Planning and Decisions Act 2016 (MTPD Act), a competent adult has the right to refuse medical treatment, even if it is not objectively in the patient's best interest or may be expected to lead to serious harm or death. Clinical staff must respect and act on the patient's decision to discharge against medical advice irrespective of their personal beliefs and opinions.
2. While it is expected that all reasonable steps are taken to inform the patient and/or their family (or carers) of the risks and consequences of discharging against medical advice. When confronted with an aggressive patient, staff need to consider the risk to themselves and not put themselves at undue risk.
3. When discharging against medical advice, engage the patient's support network in discharge discussions, including the decision to commence or reinstate appropriate community supports.
4. Reasonable attempts shall be made to provide a verbal handover, in addition to a formal discharge summary, to the patient's general practitioner and pre-existing community services.
5. Where patients lack the capacity to make informed decisions, the appropriate substitute decision Maker, as described in the MTPD Act, must be involved in the discharge conversation.
6. Where the impaired capacity is due to a treatable mental illness, compulsory treatment under the Mental Health and Wellbeing Act 2022, can be invoked, if the intention is to treat the mental illness, though the existence of a compulsory order (assessment or treatment order) does not automatically, in and of itself, imply incapacity to make treatment decisions.
7. The offence of medical trespass or false imprisonment may apply to doctors (and those working under their direction) who attempt to restrain or restrict a competent patient's means of egress.
8. If a patient detained under the Mental Health and Wellbeing Act expresses a desire to discharge, this must be treated in accordance with the Mental Health and Wellbeing Act and discussed with the Authorised Psychiatrist.
9. Patients waiting in Emergency Departments for a mental health assessment must not be Discharged against Medical Advice. There must be presumed concern about their capacity to make a rational decision and so they cannot be allowed to leave against medical advice without

Discharge against medical advice

Procedure

being assessed as to that competence by an ED Consultant or ED Medical Team Leader overnight.
If required the assessment must be expedited by the Emergency Psychiatric Service/Senior Doctor.

PROCEDURE

Patient expresses intention to discharge against medical advice.

1. Nursing staff to advise the medical team of the patient's intention to discharge against medical advice.
2. Hospital Medical Officer to discuss patient's intention to discharge against medical advice with the supervising consultant or registrar to decide on the best person to have the conversation with the patient/family or carer. The HMO proceeds to discuss the discharge against medical advice with the patient as well as the patient's family/ support network.
3. Medical staff to determine whether the patient has capacity to be able to discharge against medical advice. Capacity is to be documented in the medical records. The patient needs to demonstrate all four of the following to meet minimal requirements. The minimal requirements include:
 - a) Ability to understand information;
 - b) Ability to recall information;
 - c) Ability to weigh information and come to a conclusion; and
 - d) Ability to communicate the decision.
4. If the patient lacks capacity to make informed decisions, then all reasonable attempts to involve a carer or family member to discuss the discharge planning process becomes even more important.
5. Make all reasonable attempts to give the following information to the patient/substitute decision maker in a language they understand or using an interpreter to help them make an informed decision
 - Nature of their medical condition, the prognosis, and the treatment that is to be provided in hospital
 - Reasons for, and the benefits of, the patient remaining in hospital and receiving treatment
 - Risks of not receiving treatment in hospital
 - Seriousness of their condition and the potential complications that may ensue if treatment is not completed.
6. If the patient/family member or carer still wishes to discharge against medical advice make all reasonable attempts to do the following
 - Discuss with them a treatment plan to support and minimise the risks associated with self-discharge
 - Arrange a follow up appointment for the patient
 - Where possible, provide a verbal handover to the GP and pre-existing community services
 - Complete a discharge summary and send to the GP
 - Arrange for discharge medications to be given
 - Discuss safety of home environment
 - Explain any specific precautions that the patient may need to observe

PROMPT Doc No: SNH0021674 v6.1

Date loaded on PROMPT: 04/01/2016

Page 2 of 4

Review By: 21/06/2027

Version Changed: 14/07/2026

Document uncontrolled when
downloaded.

Last Reviewed Date: 21/03/2024

Discharge against medical advice

Procedure

- Explain that they can seek treatment at Monash Health in the future if they wish to and access the Emergency Department in an emergency
 - Document information given to the patient including risks explained in the health record
 - Complete discharge against medical advice form (MRE11) and ensure the patient signs it.
7. If patient or the family is unwilling to participate in the above conversation or sign the form document this clearly in the health record.

Discharge Against Medical Advice (DAMA) Checklist

Item	Description	Responsibility
1	Notify the medical team of patient's intention to DAMA.	Nursing staff
2	Conduct a clinical/physical review of patient: • Seek to understand the underlying reasons for request DAMA • It may be possible to address their concerns and provide the necessary support to encourage them to remain in care • When confronted with an aggressive patient, consider the risk to oneself	JMS
3	Discuss patient's intention with supervising consultant or registrar	JMS
4	Determine patient's capacity to make informed decisions and document in medical records. If the patient lacks capacity, the appropriate substitute decision Maker, as described in the MTPD Act, must be involved in the discharge conversation and, where applicable, provide information to patient/ substitute decision maker in a language they understand or with an interpreter. For further guidance, refer to the Consent and refusal for medical treatment for patients who lack decision-making capacity - Procedure	JMS/SMS
5	Make reasonable attempts to cover the following if the patient/family member or carer wishes to discharge against medical advice (if the patient or family is unwilling to participate in discussions or sign any form, document this in the health record):	JMS/SMS
5a	• Discuss a treatment plan to support and minimise risks	
5b	• Arrange a follow-up appointment	
5c	• Arrange for discharge medications to be given	
5d	• Discuss the safety of the home environment	
5e	• Explain any specific precautions the patient may need to observe	
5f	• Explain that they can seek treatment at the hospital in the future and access the Emergency Department in an emergency	
5g	• When possible, provide a verbal handover to the GP and pre-existing community services	
5h	• Complete a discharge summary and send to the GP	
6	Document discussion, assessment and information given to the patient, including risks explained, in the health record	JMS/SMS
7	Complete discharge against medical advice form (MRE11) and ensure the patient signs it	JMS/SMS
NB	1. Where the impaired capacity is due to a treatable mental illness, compulsory treatment under the Mental Health and Wellbeing Act 2022 , can be invoked, if the intention is to treat the mental illness, though the existence of a compulsory	JMS/SMS Emergency Staff

Discharge against medical advice

Procedure

	order (assessment or treatment order) does not automatically, in and of itself, imply incapacity to make treatment decisions. If a patient detained under the Mental Health and Wellbeing Act expresses a desire to discharge, ensure the process is treated in accordance with the Mental Health and Wellbeing Act and discussed with the Authorised Psychiatrist. 2. For patients waiting in Emergency Departments for a mental health assessment, ensure they are not discharged against medical advice without being assessed for capacity by an ED Consultant or ED Senior Registrar overnight.	
--	---	--

EQUIPMENT

[Determining Decision Making Capacity – Implementation Tool](#)

[Can your adult patient consent? – Implementation Tool](#)

Document Governance	
Supporting Policy	Clinical Communication (Operational)
Executive Sponsor	Chief Medical Officer
Unit Responsible	Monash Doctors Workforce
Document Author	Director Medical Services
Consumer Review Yes or No	No
This Procedure has been endorsed by an EMR Subject Matter Expert (SME)	There are no Order Set or Quick Reference Guides linked